

Patient NAME : Ms Jayanti
 DOB/Age/Gender : 26 Y/Female
 Patient ID / UHID : 13936539/RCL8057905
 Referred BY : Self
 Sample Collected : Sep 17, 2025, 10:01 AM

Report STATUS : Final Report

Barcode NO : 25812537

Sample Type : Whole blood EDTA

Report Date : Sep 17, 2025, 06:41 PM.

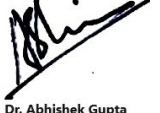


Test Description	Value(s)	Unit(s)	Reference Range
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Autoriders Camp**Complete Blood Count (CBC)**

RBC Parameters			
Hemoglobin <i>colorimetric</i>	8.7	g/dL	12.0 - 15.0
RBC Count <i>Electrical impedance</i>	3.3	10 ⁶ /μl	3.8 - 4.8
PCV <i>Calculated</i>	24.7	%	36 - 46
MCV <i>Calculated</i>	75.8	fl	83 - 101
MCH <i>Calculated</i>	26.6	pg	27 - 32
MCHC <i>Calculated</i>	35.1	g/dL	31.5 - 34.5
RDW (CV) * <i>Calculated</i>	21.2	%	11.6 - 14.0
RDW-SD * <i>Calculated</i>	59	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance and microscopy</i>	6	10 ³ /μl	4 - 10
Differential Leucocyte Count			
Neutrophils	70	%	40-80
Lymphocytes	20	%	20-40
Monocytes	7	%	2-10
Eosinophils	03	%	1-6
Basophils	0	%	<2
Absolute Leukocyte Counts <i>Calculated</i>			
Neutrophils.	4.2	10 ³ /μl	2 - 7
Lymphocytes.	1.2	10 ³ /μl	1 - 3
Monocytes.	0.42	10 ³ /μl	0.2 - 1.0
Eosinophils.	0.18	10 ³ /μl	0.02 - 0.5
Basophils.	0	10 ³ /μl	0.02 - 0.5
Platelet Parameters			
Platelet Count <i>Electrical impedance and microscopy</i>	123	10 ³ /μl	150 - 410
Mean Platelet Volume (MPV) * <i>Calculated</i>	9.8	fL	9.3 - 12.1
PCT * <i>Calculated</i>	0.1	%	0.17 - 0.32

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Processing Lab :- Redcliffe Lifetech Pvt. Ltd., 41/2, Pocket No.2, Vivek Vihar, GMS Road, Dehradun-248001

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PDW * <i>Calculated</i>	10.5	fL	8.3 - 25.0
P-LCR * <i>Calculated</i>	24.2	%	18 - 50
P-LCC * <i>Calculated</i>	30	10 ⁹ /L	44 - 140
Mentzer Index * <i>Calculated</i>	22.97	%	> 13

Note : Kindly correlate clinically.

Advice: Repeat if clinically indicated.

Interpretation:

CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Barcode NO : 25938750

Sample Type : Serum

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Test Description	Value(s)	Unit(s)	Reference Range
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Liver Function Test (LFT)

Bilirubin Total <i>Colorimetric Diazo</i>	0.4	mg/dL	0 - 1.2
Bilirubin Direct *	0.3	mg/dL	0 - 0.20
Bilirubin Indirect * <i>Calculation (T Bil - D Bil)</i>	0.1	mg/dL	0.1 - 1.0
SGOT/AST <i>IFCC without P5P</i>	45.1	U/L	up to 40
SGPT/ALT <i>IFCC without P5P</i>	30	U/L	up to 41
SGOT/SGPT Ratio *	1.5	-	-
Alkaline Phosphatase <i>IFCC</i>	264	U/L	35 - 104
Total Protein <i>Biuret</i>	6.4	g/dL	6.0 - 7.8
Albumin <i>Colorimetric</i>	2.4	g/dL	3.5 - 5.2
Globulin * <i>Calculation (T.P - Albumin)</i>	4	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * <i>Calculation (Albumin/Globulin)</i>	0.6	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * <i>ENZYMATIC</i>	63.1	U/L	5-40

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

- 1. AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
- 2. ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
- 3. Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

- 1. Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
- 2. High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
- 3. Low albumin:** Caused by poor diet, kidney, or liver disease.
- 4. High albumin:** Usually due to severe dehydration.

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Kidney Function Test (KFT)

Blood Urea <i>Urease</i>	79.1	mg/dL	16.6 - 48.5
Bun * <i>Urease</i>	36.96	mg/dL	6 - 20
Creatinine <i>Jaffe</i>	5.9	mg/dL	0.50 - 0.90
eGFR (CKD-EPI)	9.47	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio * <i>Calculated</i>	6.26		12 - 20
Urea / Creatinine Ratio * <i>Calculated</i>	13.41		25.68- 42.8
Uric Acid <i>Enzymatic colorimetric</i>	5	mg/dL	2.4 - 5.7
Calcium Serum <i>BAPTA</i>	8.2	mg/dL	8.6 - 10.0
Phosphorus <i>Molybdate UV</i>	4.6	mg/dL	2.5 - 4.5
Sodium <i>ISE-Indirect</i>	139.6	mmol/L	136 - 145
Potassium <i>ISE-Indirect</i>	2.9	mmol/L	3.5 - 5.1
Chloride <i>ISE-Indirect</i>	102.9	mmol/L	98 - 107

Note : Kindly correlate clinically.
 Advice: Repeat if clinically indicated.

Interpretation:

Kidney function tests is a collective term for a variety of individual tests and procedures that can be done to evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carry out their vital functions. Some lead to a rapid (acute) decline in kidney function, others lead to a gradual (chronic) decline in function. Both result in a buildup of toxic waste substances in the blood, as well as on urine samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure, blood in urine, frequent urges to urinate, difficulty beginning urination, painful urination, swelling in the hands and feet

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due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Electrolytes are present in the human body and the balancing act of the electrolytes in our bodies is essential for normal function of our cells and organs. There has to be a balance. Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases. "eGFR test is applicable for patients aged 18 years or more."			

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Lipid Profile

Total Cholesterol <i>Enzymatic - Cholesterol Oxidase</i>	166	mg/dL	<200
Triglycerides <i>Colorimetric - Lip/Glycerol Kinase</i>	69.5	mg/dL	<150
HDL Cholesterol <i>Enzymatic colorimetric</i>	59.2	mg/dL	>55
Non HDL Cholesterol * <i>Calculated</i>	106.8	mg/dL	<130
LDL Cholesterol * <i>Calculated</i>	92.9	mg/dL	<100
V.L.D.L Cholesterol * <i>Calculated</i>	13.9	mg/dL	< 30
Chol/HDL Ratio * <i>Calculated</i>	2.8	Ratio	3.5 - 5.0
HDL/ LDL Ratio * <i>Calculated</i>	0.64	Ratio	0.5 - 3.0
LDL/HDL Ratio * <i>Calculated</i>	1.57	Ratio	-

Interpretation:

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

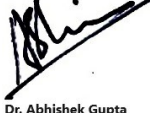
National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence

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High Risk	of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque			
Moderate Risk	2 major ASCVD risk factors			
Low Risk	0-1 major ASCVD risk factors			
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors				
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use			
2. Family history of premature ASCVD	4. High blood pressure			
5. Low HDL				

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

* After an adequate non-pharmacological intervention for at least 3 months.

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

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Barcode NO : 25925522

Sample Type : Spot Urine

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Test Description	Value(s)	Unit(s)	Reference Range
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Urine Routine and Microscopic Examination

Physical Examination			
Volume *	10	ml	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	Slightly Hazy	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination			
Reaction (pH) <i>Double Indicator</i>	6.0	-	4.5 - 8.0
Specific Gravity <i>Ion Exchange</i>	1.005	-	1.010 - 1.030
Urine Glucose (sugar) <i>Oxidase / Peroxidase</i>	Positive(+)	-	Negative
Urine Protein (Albumin) <i>Acid / Base Colour Exchahnge</i>	Positive(Trace)	-	Negative
Urine Ketones (Acetone) <i>Legals Test</i>	Negative	-	Negative
Blood <i>Peroxidase Hemoglobin</i>	Positive(++)	-	Negative
Leucocyte esterase <i>Enzymatic Reaction</i>	Negative	-	Negative
Bilirubin Urine <i>Coupling Reaction</i>	Negative	-	Negative
Nitrite <i>Griless Test</i>	Negative	-	Negative
Urobilinogen <i>Ehrlichs Test</i>	Normal	-	Normal
Microscopic Examination			
Pus Cells (WBCs) *	25-30	/hpf	0 - 5
Epithelial Cells *	8-10	/hpf	0 - 4
Red blood Cells *	70-80	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	Present	-	Absent
Protozoa *	Absent	-	Absent
Interpretation: URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders. Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous			

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exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications. Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise. Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders. Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection. Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection. pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine. Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus. Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine. Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.			

*** End Of Report ***

Pending Report To Follow - HbA1C

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